

7.35.3 NMAC: proposed amendments outside the practicum. 3 of 4, Locations and oversight

Working draft v1, July 26, 2026. Rule hearing August 28, 2026. Not a filing, not submitted, and not adopted rule text.

- 7.35.3.11, .20 and .21. Healing center and other approved location applications, the registration on which healing center staff authority depends, and department assessments.
- This is one of four documents covering the twenty-four sections of 7.35.3 NMAC that the practicum amendment draft in *amendments/* does not reach. That draft covers 7.35.3.14, .18, .19, Paragraph (5) of Subsection H of .20, and a proposed .29, and nothing here reopens any of them.
- Every proposed change carries a citation to the provision, source document, or transcript it comes from. Where a defect has no fix that a source supplies, the published text is left exactly as it stands and the question is stated at that provision, with the choices and who decides.
- No figure appears in a right column unless a source carries it. A figure taken from elsewhere in the rule is badged with where it comes from.

The columns. Left is the rule as published, verbatim. Right is the proposed amendment: underlined green inserted, ~~struck red deleted~~, unmarked text carried forward unchanged. 60 from 7.35.3.15 B marks a figure taken from another provision of the same rule rather than newly drafted. A provision shown with no amendment is reproduced because it carries a finding.

7.35.3.11 Application process for healing centers and other approved locations

3 OF 4 · LOCATIONS AND OVERSIGHT

CURRENT LANGUAGE	PROPOSED AMENDMENT
Proposed rule as published July 23, 2026	Draft for review. Not filed, not adopted
Section heading and lead-in of Subsection A 7.35.3.11 APPLICATION PROCESS FOR HEALING CENTERS AND OTHER APPROVED LOCATIONS: (A) Applications for healing centers: An applicant seeking certification as a healing center shall submit a complete application through the electronic system designated by the department. A complete application shall include, at minimum, the following:	Section heading and lead-in of Subsection A 7.35.3.11 APPLICATION PROCESS FOR HEALING CENTERS AND OTHER APPROVED LOCATIONS: (A) <u>A</u>. Applications for healing centers: An applicant seeking certification as a healing center shall submit a complete application through the electronic system designated by the department. A complete application shall include, at minimum, the following: SOURCE Correction only. 7.35.3.11 NMAC Subsection A, page 5, is lettered "(A)"; Subsections B, C and D of the same section use the form "B.", "C." and "D." The only other section of the rule that uses parenthetical subsection letters is 7.35.3.14 NMAC, page 9, which is addressed in the practicum amendment document. No substance is changed.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Paragraph (11) of Subsection A, owners and employees

(11) A list of all owners or members of the board of directors, including corresponding contact information;

Paragraph (11) of Subsection A, owners and employees

(11) A list of all owners or members of the board of directors, including corresponding contact information; and a list of all owners and employees whom the healing center designates to purchase, possess, sell, or otherwise administer medical psilocybin products under Subsection C of 7.35.3.14 NMAC, including corresponding contact information. An owner or employee named on the list submitted under this paragraph is registered with the department for purposes of Subsection C of 7.35.3.14 NMAC;

SOURCE Finding B5 of `analysis/july23-rule-concerns.md`, verified against the published rule. Subsection C of 7.35.3.14 NMAC, page 9, conditions the authority of healing center owners and employees to purchase, possess, sell or administer medical psilocybin on their being "registered with the department". The phrase "registered with the department" appears once in 7.35.3 NMAC, at that provision, and no registration for owners or employees exists in 7.35.3 NMAC or in 7.35.2 NMAC. The amendment supplies the registration at the point where the healing center already files the names: Paragraph (11) of Subsection A already requires "A list of all owners or members of the board of directors, including corresponding contact information", Paragraph (2) of the same subsection already requires "Names and contact information of primary contact person(s) and any affiliated practitioners or facilitators", and Subsection C of 7.35.3.14 NMAC already requires that the individual be "designated to engage in each activity by the healing center". Subsection C of 7.35.3.14 NMAC is new in the published rule; the July 9, 2026 draft contained no authorized-possession section. No figure is added.

Please review. The registration drafted here operates on the list the healing center files with its application, which fixes the position as at the date of the application. Nothing in 7.35.3.11 NMAC requires a healing center to update that list, and 7.35.3.11 NMAC has four subsections, A through D, none of which is a reporting duty; the only comparable duty in the rule is Subsection A of 7.35.3.15 NMAC, page 9, which applies to educational programs and not to healing centers. So an owner or employee who joins after certification would not be registered until the renewal under Subsection C of this section, which falls at two years. Three choices. Add a reporting duty for changes to the list, which is a new obligation that no source in this record proposes and which is therefore not drafted here. Rely on the two-year renewal. Or strike the registration condition from Subsection C of 7.35.3.14 NMAC instead of supplying a registration, which would leave the designation by the healing center as the only requirement. Department of Health staff decide, and the third choice has to be coordinated with the practicum amendment document because Subsection C of 7.35.3.14 NMAC is in that draft's scope. Whichever is chosen, the condition in Subsection C of 7.35.3.14 NMAC cannot be satisfied as published.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Paragraph (9) of Subsection B, proof of ownership

(9) Proof of ownership by the qualified patient or attending practitioner or facilitator of any property on which medical psilocybin administration sessions will be conducted, or a signed, written statement from the owner of such property acknowledging that the owner understands that persons will be participating in the medical psilocybin program on the premises, and what those persons are authorized to do within the terms of their certification;

Paragraph (9) of Subsection B, proof of ownership

(9) Proof of ownership by the qualified patient or attending practitioner or facilitator of any property on which medical psilocybin administration sessions will be conducted, or a signed, written statement from the owner of such property acknowledging that the owner understands that persons will be participating in the medical psilocybin program on the premises, and what those persons are authorized to do within the terms of their certification. **This paragraph does not apply where the location is the residence of the qualified patient;**

SOURCE Wilson working redline of July 25, 2026, comment on this paragraph: "seems like this will require EOL patients who are getting at home treatment to get permission from their landlord. We don't require that for any other in home healthcare and it's not appropriate to require that for EOL patients. People should not have to get their landlords permission to have certain healthcare." End-of-life care is a qualifying condition under Section 3(l) of the Medical Psilocybin Act and under 7.35.2.7 NMAC. 7.35.3.11 NMAC Paragraph (9) of Subsection B, page 6. Carried over from the July 9, 2026 draft.

Please review. The amendment exempts the qualified patient's own residence from the ownership proof and the owner's written statement. Two things to weigh. Subsection B of this section makes an other approved location available in part "for purposes of improving patient access in rural and frontier counties", and a patient's residence is the leading example the same subsection gives. Against that, the owner's statement is the only place in the rule where a property owner is told what will occur on the premises, so exempting the residence removes that notice for the location where sessions are most likely to be unsupervised by a healing center. Two choices. Adopt the exemption as drafted, or keep the requirement and allow the patient's own attestation in place of the owner's statement. The board decides, because the trade is patient access against notice to a third party.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Paragraph (10) of Subsection B, outdoor or natural environments

(10) For outdoor or natural environments: (a) A detailed description of the administration area, including identification of safe entrances and exits, and verification that the area is free from hazards; (b) An emergency safety and response plan; (c) Proof that emergency medical services can reliably be contacted from, and respond to, the location at which medical psilocybin administration sessions will occur; and (d) If the natural environment setting is 15 minutes or more away from emergency medical service response or a hospital, urgent care, or other emergency medical services, then there must be at least two individuals present who are not receiving treatment and who have: (i) Wilderness first aid certification; (ii) Wilderness first responder certification; or (iii) Licensure as a New Mexico emergency medical technician (e.g., EMT-basic, EMT-intermediate, EMT-paramedic);

Paragraph (10) of Subsection B, outdoor or natural environments

(10) For outdoor or natural environments: (a) A detailed description of the administration area, including identification of safe entrances and exits, and verification that the area is free from hazards; (b) An emergency safety and response plan; (c) Proof that emergency medical services can reliably be contacted from, and respond to, the location at which medical psilocybin administration sessions will occur; and (d) If the natural environment setting is 15 minutes or more away from emergency medical service response or a hospital, urgent care, or other emergency medical services, then there must be at least two individuals present who are not receiving treatment and who have: (i) Wilderness first aid certification; (ii) Wilderness first responder certification; or (iii) Licensure as a New Mexico emergency medical technician (e.g., EMT-basic, EMT-intermediate, EMT-paramedic); **and (e) If the natural environment setting is 15 minutes or more away from emergency medical service response or a hospital, urgent care, or other emergency medical services, then there must be a basic first aid kit and an AED on site;**

SOURCE Conforming amendment. Finding M16 of `analysis/july23-rule-concerns.md`, verified against the published rule. The outdoor requirements for a healing center at 7.35.3.11 NMAC Paragraph (22) of Subsection A, pages 5 and 6, run to item (e) and require “a basic first aid kit and an AED on site”. The outdoor requirements for an other approved location at Paragraph (10) of Subsection B of the same section, pages 6 and 7, stop at item (d). The added item is the text of item (e) of Paragraph (22) of Subsection A, reproduced. No figure is added; the 15 minutes is the figure already in both paragraphs.

Please review. Two further questions on the same paragraph, neither answered here.
The 15 minutes: the Wilson working redline comments on both places the figure appears, “Is this a reasonable limit? Everything feels at least 15 minutes away - should we say 30?” and “again - I think this should maybe be 30. Everything is 15 minutes away from everything.” Those are questions rather than a recommendation, and no source in this record supplies a figure, so the 15 minutes is left as published in both paragraphs. The two-person requirement: the redline adds at the healing center paragraph that “For the purposes of this section, an individual is a DOH-permitted practitioner, a DOH-permitted facilitator, or an individual who is a DOH registered student with an approved training program who has completed a minimum of 30 practicum hours and who is accompanied by a DOH-permitted practitioner.”, and the comment on it reads “Not sure where this goes - the 2 person universal requirement did not make it into the published rules, though DOH did confirm they mean for it to be universal.” Because the author records that she does not know where the provision belongs, it is not drafted. Ms. Wilson and Department of Health staff decide where it goes and whether the 15 minutes becomes 30.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Subsection D, certification period; approval and denial

D. Certification period; approval and denial: Certification of a healing center shall be effective on the date of department issuance and shall be valid for two years. Certification of other approved locations shall be effective on the date of department issuance and shall be valid for 90 days. If the department denies an application for a healing center or other approved location, the department shall provide notice of the denial within 30 calendar days in accordance with this rule. If the application is denied, the applicant may re-apply within six months. An applicant who is denied a second time may not re-apply for six months from the denial. An applicant whose application is denied may appeal the denial in accordance with this rule.

Subsection D, certification period; approval and denial

D. Certification period; approval and denial: Certification of a healing center shall be effective on the date of department issuance and shall be valid for two years. Certification of other approved locations shall be effective on the date of department issuance and shall be valid for 90 days, and may be renewed for additional 90-day terms on application to the department **90 from 7.35.3.11 D**. If the department denies an application for a healing center or other approved location, the department shall provide notice of the denial within 30 calendar days in accordance with this rule. If the application is denied, the applicant may re-apply ~~within no~~ earlier than six months after the date of the denial. An applicant who is denied a second time may not re-apply for six months from the denial. An applicant whose application is denied may appeal the denial in accordance with this rule.

SOURCE Two changes. Renewal: finding M22 of `analysis/july23-rule-concerns.md`, verified against the published rule. Subsection D gives an other approved location a 90-day certification and no renewal route, while Subsection C of the same section supplies a renewal route for healing centers only. The 90 days in the amendment is the figure already in Subsection D. Re-application: the same conforming change as 7.35.3.10 NMAC Subsection C. 7.35.3.11 NMAC Subsection D, page 7. Carried over from the July 9, 2026 draft.

Please review. The renewal drafted here repeats the 90 days the subsection already sets, so nothing new is introduced, but three questions remain. Whether 90 days is the right term at all, which no source in this record addresses. Whether a renewal application needs its own contents, or whether the application under Subsection B of this section is resubmitted. Whether the department is bound to any period in deciding a renewal, since Subsection D binds it to 30 calendar days only for a denial. Department of Health staff decide all three, because each one binds the department, and the Training and Education Committee should say whether a 90-day term with renewal is workable for a practicum site.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading, lead-in, and Subsection A**7.34.3.20 REQUIREMENTS FOR HEALING CENTERS AND OTHER APPROVED**

LOCATIONS: A healing center and a registrant of another approved location shall comply with the following requirements: A. A healing center and a registrant of another approved location shall maintain a list of all qualified patients who are intended to utilize, and who have utilized, the location for administration sessions.

Section heading, lead-in, and Subsection A

Not amended. This provision is reproduced because it carries a finding recorded below.

SOURCE Not amended here. The section heading reads 7.34.3.20 and its correction is drafted in the practicum amendment document, so it is not duplicated. Recorded for two other things. Terminology: the lead-in and Subsection A, page 13, use “a registrant of another approved location”, and the term “registrant” appears 10 times in 7.35.3.20 NMAC and nowhere else in the rule, while Subsection B of 7.35.3.11 NMAC, page 6, issues a certification for such a location rather than a registration; the definition drafted at 7.35.3.7 NMAC ties the two together. Patient roster: finding M18. Carried over from the July 9, 2026 draft.

Please review. Two questions, neither drafted here. Terminology: this section calls the holder of an other approved location certification a registrant 10 times, and Subsection B of 7.35.3.11 NMAC issues a certification. Two choices. Adopt the definition drafted at 7.35.3.7 NMAC, which ties the two words together and leaves this section untouched, or conform all 10 occurrences here to certificant. Department of Health staff decide which, and the second choice requires 10 edits this document does not make. Patient roster: finding M18 of `analysis/july23-rule-concerns.md` records that Subsection A requires a list of all qualified patients intended to use and who have used the location, which read with Subsection A of 7.35.3.21 NMAC allows the department to obtain patient identities and interview patients. Whether the roster should instead be kept in a form that does not identify patients to the department is a question for the board, and it turns on the department's data needs, which Department of Health staff would have to state.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Subsection A, authority to conduct assessments

A. Authority to conduct assessments: The department or its designee may conduct remote or on-site assessments of a psilocybin education program, healing center or other approved location, with or without prior notice, during normal business hours, for the purpose of determining compliance with the Medical Psilocybin Act and this rule. In conducting assessments, the department may review documentation, inspect premises and equipment, and interview staff and patients. Exception: except in emergency circumstances, the department or its designee shall not conduct an on-site assessment during a patient session or at a patient's home without prior written consent from the patient(s).

Subsection A, authority to conduct assessments

Not amended. This provision is reproduced because it carries a finding recorded below.

SOURCE Not amended. Recorded for two things. Patient interviews: Subsection A, page 15, permits the department to "interview staff and patients", subject to the exception in the same subsection for an on-site assessment during a patient session or at a patient's home. Frequency: 7.35.3.21 NMAC sets no interval at which assessments occur, while 7.35.3.16 NMAC Subsection E, page 10, requires a third-party evaluation report without stating how often. Carried over from the July 9, 2026 draft, where the section was titled "PSILOCYBIN EDUCATIONAL PROGRAMS, HEALING CENTERS, OTHER APPROVED LOCATIONS; EVALUATION AND ASSESSMENT BY THE DEPARTMENT".

Please review. Two questions, neither drafted here. Patient interviews: the exception in this subsection protects a patient during a session and at home, and does not otherwise limit whom the department may interview or require that the patient consent. Whether an additional consent requirement should apply to any patient interview is a question for the board. Frequency: the rule does not say how often the department assesses a program, a healing center or an other approved location, and it does not say how often a third-party evaluation must be repeated once the initial one is filed under Paragraph (20) of Subsection A of 7.35.3.12 NMAC. Finding M10 of `analysis/july23-rule-concerns.md` records the second point. No source in this record supplies an interval, so none is drafted. Department of Health staff decide both, because each is a demand on department capacity.

Addendum A Terms this part uses and no part defines

7.35.3.7 NMAC provides in full: "The definitions in 7.35.2.7 NMAC apply to this part." Every count in the table below is produced from the text layer of the published rule and of 7.35.2 NMAC each time this document is built, section by section, so no count in it is transcribed by hand. Column headings are section numbers within each part. The first block is terms used in 7.35.3 NMAC that 7.35.2.7 NMAC does not define. The second block, shaded, is the terms 7.35.2.7 NMAC does define, shown for contrast.

Addendum A

TERMS THIS PART USES AND NO PART DEFINES

Term in 7.35.3 NMAC	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	25	26	27	28	Part 3	Part 2
educational program	1					1				28		7			3	3	13	1	2	1	3					1	3		68	0
facilitator	1					1			5	5	3	1	14	5			2	15	4	3						1	4		65	0
healing center	1										10	1	2	10					1	21	3					1	3		54	0
certifying clinician						1		16	5	4			11					10								1	4		53	0
other approved location	1										6		1	3					1	14	3					1	3		34	0
practicum									3	4		1			1		5		13	2									29	0
student												5				3	12		5	3									28	0
certificant																					2	3	3	2			1		11	0
registrant																				10									10	0
didactic										4								4	2										10	0
adverse health event											1		1							4									6	0
administrative review committee																									4				4	0
graduate												1					2		1										4	0
simulated patient																	1	2	1										4	0
medical psilocybin services													1					1		1									3	0
equity and access fund								1																					1	0
Defined in 7.35.2.7 NMAC, for contrast																														
practitioner	1					1			5	5	3	1	12	6			2	14	6	3						1	5		66	12
clinician	1						5						4												1		2		13	4
qualified patient						1		3			2		4	3						3		3		1		1	1		22	6
administration session											5			3				1		7		1							17	4
approved location	1					1					6	1	1	3					1	15	3					1	3		37	2
guide																													0	3

What the table shows. Sixteen terms carrying regulatory consequence in this part are defined in neither part. Four of them are defined in the draft at 7.35.3.7 NMAC, because a single provision of the published rule supplies the content: certificant, facilitator, registrant of another approved location, and student. Twelve are not, and the two largest are healing center and certifying clinician. "Guide", the one defined term in 7.35.2.7 NMAC for an individual who assists practitioners during administration sessions, appears nowhere in 7.35.3 NMAC, while "facilitator", which 7.35.2.7 NMAC does not define, appears throughout it. 7.35.2 NMAC was adopted effective June 23, 2026, so amending 7.35.2.7 NMAC is a separate rulemaking and any term this part needs has to be carried in this part.

The Act. Section 3(B) of the Medical Psilocybin Act defines a clinician as an approved health care provider licensed in New Mexico who holds a permit from the department to provide medical services to qualified patients. 7.35.2.7 NMAC defines "Clinician" in the same terms except that it reads certification where the Act reads permit, and defines "Permit" as the authorization to operate as a psilocybin producer or psilocybin testing laboratory. Section 5 of the Act exempts a producer, a clinician and a qualified patient from arrest, prosecution and penalty. Neither the Act nor 7.35.2.7 NMAC uses the word facilitator. The consequence is stated at 7.35.3.7 NMAC in the redline above.

Addendum B Dependencies across the two drafting scopes

Provisions in these four documents that operate on, or are operated on by, a provision the practicum amendment draft in *amendments/* reaches. Nothing in these documents amends a provision in that draft's scope. Where a fix would require one, it is recorded here and left undrafted.

Addendum B

DEPENDENCIES ACROSS THE TWO DRAFTING SCOPES

Provision here	Provision in the practicum draft's scope	Relationship	Handled
7.35.3.11 A(11), page 5	7.35.3.14 C, page 9	7.35.3.14 C conditions healing center owner and employee authority on being registered with the department, and no registration exists. The registration is drafted here, at the point where the healing center already files the names.	Drafted here. The practicum draft flagged 7.35.3.14 C and did not amend it, and it is not amended here either.
7.35.3.7, page 1	7.35.3.20 H(5), page 14	The definition of student drafted here is taken from the qualified student test in 7.35.3.20 H(5). The practicum draft amends that paragraph for the permit title and does not change the test.	Drafted here, sourced there. If the test changes, the definition follows it.
7.35.3.7, page 1	7.35.3.14 B, page 9	7.35.3.14 B authorizes facilitators to possess psilocybin products and provide them to qualified patients. Whether a facilitator is a clinician under Section 3(B) of the Medical Psilocybin Act, and so within the Section 5 exemption, governs whether that authority holds. This is the administering half of Paragraph (2) of Subsection B of Section 5 of the Act. The practicum draft reads the taking half of the same provision against Subsection H of Section 3, and its proposed 7.35.3.29 states the statutory amendment that half would need and conditions the section on it, so the two drafts read the same provision consistently.	Not drafted. Recorded at 7.35.3.7 as a question for department counsel.
7.35.3.7, page 1	7.35.3.14 A and C, page 9, with 7.35.3.20 D, page 14	The position of a student is not in doubt and is not raised in these documents. The practicum draft locates the provision of the medicine in the practitioner or the healing center, and 7.35.3.20 D provides that "students completing their practicums may be present during an administration session". The definition of student drafted here does not disturb that.	Settled in the practicum draft. Nothing here contradicts it.
7.35.3.13 B, page 8	7.35.3.14 B, 7.35.3.19 C, 7.35.3.20 H(5)	The facilitator scope of work in 7.35.3.13 B is narrower than the authority the other three provisions confer, and "direct supervision" is not defined anywhere in the rule.	Not drafted. Any amendment has to be coordinated with the practicum draft.
7.35.3.17 A, pages 10 to 11	Proposed 7.35.3.19 H, which exists only in that draft's re-lettering of 7.35.3.19	Whether the consultation requirement is stated in hours or in cases, and whether it sits at the proposed 7.35.3.19 H or at 7.35.3.17 A, is open in the practicum document for Dr. Metz, Ms. Wilson and Dr. Leeman. As published, 7.35.3.19 NMAC runs A through G and has no Subsection H.	Not drafted. 7.35.3.17 A is left exactly as published so that their answer governs.
7.35.3.10 D(1), page 5	7.35.3.19 G(4), page 13, which that draft re-letters as 7.35.3.19 K	Two waivers of the practicum hours requirement each set a 40-hour floor for applications received by December 31, 2027. For the same 40 hours of contact time, 7.35.3.10 D(1) requires "two separate group sessions" and 7.35.3.19 G(4) requires "A minimum of one group session". The practicum draft records the conflict at its 7.35.3.19 K and amends neither provision, on the ground that 7.35.3.10 is outside its scope.	Not drafted here either. Conforming one to the other is a choice between two figures already in the rule, so neither document makes it and the conflict stands until the committee chooses.
7.35.3.16 A and C, page 10	7.35.3.18, pages 11 to 12	The third-party evaluation assesses the curriculum that 7.35.3.18 requires. Curing the evaluator conflict does not touch the curriculum, and the practicum draft's changes to the curriculum do not touch the conflict.	Drafted here. No coordination needed.
7.35.3.20 heading, page 13	7.35.3.20 heading, page 13	The heading reads 7.34.3.20. The practicum draft corrects it.	Not duplicated here. The correction stands in the practicum draft.

Addendum C Defects recorded and not drafted, and why

Every defect this record found in the twenty-four sections, that this draft does not amend. The reason in each row is the reason no language is proposed, not an assessment of severity. Each has a review note at the provision named, stating the choices and who decides.

Addendum C

DEFECTS RECORDED AND NOT DRAFTED, AND WHY

Provision	Defect	Why nothing is drafted
7.35.3.5, page 1, with the history notes at page 19	Twenty-six history notes carry the placeholder "xx/xx/2026"; the notes for 7.35.3.27 and 7.35.3.28 carry 9/22/2026. Under 7.35.3.5 a later date cited at the end of a section controls, so two sections would take effect before the rest has a date.	No source supplies the intended effective date, and which way to conform is the department's to choose.
7.35.3.7, page 1	Twelve of the sixteen undefined terms have no definition in any source, including healing center, 54 occurrences, and certifying clinician, 53 occurrences.	Drafting a defined term with regulatory consequence from no source would be composing it. Addendum A records the slate.
7.35.3.9 B, page 3	A renewal application is due no less than 30 days before expiry, and the department has no deadline to decide it, so a certification can lapse while a timely renewal is pending. Nothing continues the certification in the interim.	The fix is a continued-validity provision, which is a new grant, and no source in this record proposes its terms.
7.35.3.9 E(1) and F, page 3	A practitioner applicant must document a professional license "(e.g. PSY, LSW, LCSW)" with no stated scope of practice, and a facilitator applicant needs no license at all. The Wilson working redline comments "Also no scope here - what happened to therapy scope? It's missing - if DOH wants that, they need to include it." and "without specifying, they're setting themselves up to have athletic trainers and massage therapists apply and then not have a regulation to point to explain why that's not sufficient."	The redline raises the gap and supplies no list of qualifying licenses, and no other source in this record supplies one.
7.35.3.8 C, page 2, with 7.35.3.9 A(1), page 2	The department has 30 business days to decide a patient application and 30 calendar days to decide a certificant application.	Conforming one to the other is a choice between two figures already in the rule, and both bind the department.
7.35.3.10 A(2) and A(3), pages 4 to 5	The list of out-of-jurisdiction programs populates only when an individual application relying on a program is approved, so it is empty at launch. A(3) refers to programs approved by Oregon and Colorado "as of December 31, 2027", which is after the December 31, 2027 waiver in D(1) has closed.	Both fixes are new grants of authority to the department, and no source in this record proposes their terms.
7.35.3.10 D(1), page 5, with 7.35.3.19 G(4), page 13	The two 40-hour waivers set different group-session minimums, two against one.	Conforming one to the other is a choice between two figures already in the rule. The practicum draft records the same conflict and amends neither provision, so neither document makes the choice. Addendum B.
7.35.3.11 A(22)(d) and (e), and B(10) (d), pages 5 to 7	The 15-minute threshold for a remote natural environment. The Wilson working redline asks twice whether it should be 30.	A question in the source, not a recommendation, and no source supplies a figure.
7.35.3.11, page 5	The two-person universal requirement the Wilson working redline adds, which its author records did not reach the published rule.	The author records that she does not know where the provision belongs. Placing it would answer her question.
7.35.3.12 A(20) and B(1), pages 7 to 8	An application filed on or before December 31, 2027 may be approved without the third-party evaluation provided the evaluation is submitted by December 31, 2027, so the deferral shortens to nothing as that date approaches and is unavailable on the last day.	The fix is a period, and no source in this record supplies one. Tying it to the certification term would be a choice about how long a program may operate unevaluated.
7.35.3.12, pages 7 to 8, against the July 9, 2026 draft	The grounds for denying an educational program application present in the July 9 draft do not appear in the published rule. Finding M9.	Restoring deleted grounds is a substantive policy decision, and this record cannot show whether the deletion was deliberate.

Addendum C

DEFECTS RECORDED AND NOT DRAFTED, AND WHY

Provision	Defect	Why nothing is drafted
7.35.3.17 A, pages 10 to 11	The mentoring obligation, and whether it is stated in hours or cases and where it sits.	Open in the practicum document for three named people. Addendum B.
7.35.3.17 B, page 11	The test-out price cap is a fraction of the price of "each of the educational modules", which assumes per-module pricing. The Wilson working redline strikes the subsection.	Whether the option survives at all is the committee's decision, and a restated cap is worth drafting only if it does.
7.35.3.20 A, page 13, with 7.35.3.21 A, page 15	Healing centers must keep a roster of all qualified patients intended to use and who have used the location, and the department may interview patients.	Whether the roster should be kept in a form that does not identify patients turns on the department's data needs, which no source in this record states.
7.35.3.20, page 13	The term "registrant of another approved location", 10 occurrences, against the certification that 7.35.3.11 B issues.	The definition drafted at 7.35.3.7 ties the two words together. Conforming all 10 occurrences instead is a drafting choice, and this document does not make it.
7.35.3.21, page 15	No interval is set for department assessments, and none for repeating a third-party evaluation after the first.	An interval is a figure and a demand on department capacity. No source supplies either.
7.35.3.25 C and D(2), page 16	The administrative review committee decides a denied patient application and is constituted nowhere in 7.35.3 NMAC or 7.35.2 NMAC. Four occurrences, all in this section.	Only the department can say who the committee is.
7.35.3.25 E, page 16	"Except as otherwise provided by law, there shall be no right to judicial review of a decision by the administrative review committee." The Wilson working redline comments on it.	Whether the provision is within the department's authority is a legal question for department counsel.
7.35.3.24, page 15	The July 9, 2026 draft allowed a complaint from patients or staff; the published section allows one from a qualified patient or certificant, so staff who are neither have no route.	Restoring standing for staff is a policy decision. The confidentiality assurance the July 9 draft carried is restored in the redline above, because that text exists in a source.
7.35.3.27, pages 16 to 19	A certificant suspended immediately under Subsection A can be out of practice more than 135 days before a final decision, and there is no expedited track. Finding N5.	An expedited schedule is a set of periods binding the department and the secretary, and no source supplies them.
7.35.3.8 D, 7.35.3.25 A, 7.35.3.27 C(7), pages 2, 16 and 17	Three provisions describe review of a denial and their scopes overlap, so a patient applicant denied on the merits may have both an informal review and a hearing. Finding M23.	The redline fixes the name of the proceeding only. Which track applies to which denial is a question of what process is owed.

Out of scope by decision. The controlled-substance number requirement for certifying clinicians. It appears at Paragraph (3) of Subsection B of 7.35.3.8 NMAC, page 1, and at Paragraph (2) of Subsection D of 7.35.3.9 NMAC, page 3. Paragraph (2) is reproduced in the redline above because the renumbering of the paragraphs around it required it. It is reproduced as published and is not amended, and nothing is proposed about it.

Addendum D Mechanical defects across all twenty-eight sections

Numbering, dates and drafting form, checked across the whole of 7.35.3 NMAC rather than only the sections these documents amend. Every count and every entry is produced from the text layer of the published rule each time this document is built.

Addendum D

MECHANICAL DEFECTS ACROSS ALL TWENTY-EIGHT SECTIONS

Item	Count	Where	Corrected
Section headings reading 7.34.3 instead of 7.35.3	5 of 28	7.34.3.13, 7.34.3.14, 7.34.3.20, 7.34.3.23, 7.34.3.25	7.35.3.13 in the document 4 above; 7.35.3.14 in the practicum draft; 7.35.3.20 in the practicum draft; 7.35.3.23 in the document 4 above; 7.35.3.25 in the document 4 above
History notes carrying a real effective date rather than a placeholder	2 of 28	7.35.3.27 reads 9/22/2026, 7.35.3.28 reads 9/22/2026	Not corrected. Recorded at 7.35.3.5 above.
History notes omitting the "- N" new-section designator	9 of 28	7.35.3.11, 7.35.3.13, 7.35.3.14, 7.35.3.20, 7.35.3.22, 7.35.3.23, 7.35.3.24, 7.35.3.25, 7.35.3.26	Not corrected. Mechanical, and the designator is the department's to set.
Numbering gap inside a subsection	1	7.35.3.9 D runs (1), (2), (4), (5), (6). There is no (3).	Renumbered in document 4 above.
Subsections lettered "(A)" rather than "A."	4	7.35.3.11 A, and 7.35.3.14 A, B and C. Letters found: A, B, C	7.35.3.11 A in document 3 above. 7.35.3.14 in the practicum draft.
Sentence with no verb governing the decision	1	7.35.3.25 D(1), page 16. Carried over from the July 9, 2026 draft.	Corrected in document 4 above.
Sentence that does not complete	1	7.35.3.18 F, page 12.	In the practicum draft's scope. Corrected there.
Presiding official named inconsistently	1	7.35.3.27 M reads "hearing examiner"; the rest of 7.35.3.27 reads "hearing officer".	Corrected in document 4 above.
Duplicated words	1	7.35.3.24 reads "the electronic system designated by the department electronic system".	Corrected in document 4 above.
Spacing inside a heading	1	7.35.3.16 C reads "Conflict of -interest prohibitions". The July 9, 2026 draft read "Conflict-of-interest prohibitions".	Corrected in document 2 above.
Class named in the lead-in but not in the operative words	1	7.35.3.27 B(8) opens "for certifying clinicians and practitioners" and then refers only to "the clinician".	Corrected in document 4 above.
Patient described as certified rather than enrolled	1	7.35.3.27 C(1) reads "a certified patient".	Corrected in document 4 above.

Sources. Rule as published: *docs/documents/rules-draft-2026-07-23-published.pdf*, 19 pages, 7.35.3.1 through 7.35.3.28. Prior board-meeting draft, used for new against carried-over determinations: *docs/documents/rules-draft-2026-07-09.pdf*. Defined terms: 7.35.2 NMAC as adopted effective June 23, 2026, at *source-text/7.35.2-NMAC-adopted-2026-06-23.txt*. Statute: the Medical Psilocybin Act, Senate Bill 219, 57th Legislature, First Session, 2025, as passed; Section 1 provides that Sections 1 through 11 of the act may be cited as the Medical Psilocybin Act, and those section numbers are the ones cited here. This record does not verify any NMSA 1978 section number. Working redline: Denali Wilson, NMAC 7.35.3, July 25, 2026, tracked changes and comments as exported from the document file. July 17, 2026 transcripts, morning Advisory Board and afternoon Training and Education Committee, both labeled "UNOFFICIAL AUTO-GENERATED TRANSCRIPT. NO SPEAKER ATTRIBUTION."; no speaker is named from either transcript in this document. Inventory relied on and re-verified: *analysis/july23-rule-concerns.md*.

Method. The left column reproduces the rule as published, verbatim, with line breaks introduced by the PDF collapsed to single spaces and hyphenation introduced by a line break rejoined. Nothing else is

altered. Every block was checked against the text layer of the published rule by exact contiguous match, and the build aborts if any block fails. *amendments-remainder/audit.py* re-checks every verifiable claim in these documents and exits non-zero on any failure; *amendments-remainder/AUDIT.md* is its output.

Status. Working draft v1, document 3 of 4. Not a filing, not submitted, and not adopted rule text. Nothing in *amendments/* or *docs/* was modified by this work.